

July
2024



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Client Communication

Medicare Local Coverage Determination Policy

Coverage Policy

L35396: Biomarkers for Oncology

CPT: 81120, 81121, 81170, 81175, 81176, 81206, 81207, 81208, 81210, 81218, 81219, 81233, 81235, 81236, 81237, 81245, 81246, 81261, 81262, 81263, 81270, 81272, 81273, 81275, 81276, 81287, 81292, 81293, 81294, 81301, 81305, 81310, 81311, 81313, 81314, 81315, 81316, 81320, 81321, 81322, 81323, 81327, 81334, 81340, 81342, 81345, 81347, 81348, 81351, 81352, 81353, 81357, 81360, 81435, 81436, 81437, 81438, 81445, 81450, 81479, 81503, 81520, 81525, 81538, 81540, 81546, 88120, 88121

Revision Effective Date: 12/13/2020

Coverage Indications, Limitations, and/or Medical Necessity

Compliance with the provisions in this policy may be monitored and addressed through post payment data analysis and subsequent medical review audits.

Indications

MOLECULAR TESTS

Covered clinical types of application(s) are identified below as diagnostic (DX), prognostic (PROG) or predictive (PRED).

1. Colorectal Cancer

-
- KRAS (12/13) - PRED of resistance to an anti-EGFR agent
- KRAS codon 61 - PRED of resistance to an anti-EGFR agent
- KRAS codon 146 - PRED of resistance to an anti-EGFR agent
- NRAS - PRED of resistance to an anti-EGFR agent
- BRAF - PRED of resistance to an anti-EGFR agent + DX (sporadic vs. Lynch syndrome)
- PIK3CA - PRED of resistance to an anti-EGFR agent + PROG for local recurrence
- MSI by PCR - PRED of 5-FU resistance + DX
- MLH1 promoter hypermethylation - PRED of 5-FU resistance + DX
- mRNA (oncotype-Colon) – PRED for the recurrence risk for patients with Stage II colon cancer
- Hereditary colon cancer disorders
- Sept9

ColonSeq®

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This testing provides information to the patient and provider regarding potential treatment options and implications for RAS and BRAF mutations.

Please refer to DA52986-Billing and Coding: Biomarkers for Oncology regarding coding and billing information.

2. Non-Small Cell Lung Cancer (NSCLC)

- EGFR- PRED of anti-EGFR response
- KRAS (12/13) - PRED of anti-EGFR resistance
- KRAS codon 61 - PRED of anti-EGFR resistance
- KRAS codon 146 - PRED of anti-EGFR resistance
- BRAF - PROG + PRED for anti-RAF inhibitor

ThermoFisher Oncomine DX Target Test for Non-Small Cell Lung Cancer (NSCLC) is a 23 gene panel including a 3 gene target test (companion test) approved by the FDA in June 2017 for NSCLC from tissue specimens. It can simultaneously identify the three gene variants that are a key to targeted therapy selection: BRAF and ROS1, and EGFR. The targeted therapies are dabrafenib (Tafinlar) in combination with trametinib (Mekinist), crizotinib (Xalkori), and gefitinib (Iressa), respectively. These three drugs are approved therapies for NSCLC patients with the above gene variants. Oncomine DX Target Test is the only FDA approved companion test that detects ROS1 fusions and that detects BRAF V600E, but it does not detect ALK fusions. Coverage is limited as specified in NCD 90.2, Next Generation Sequencing (NGS) for Patients with Advanced Cancer. Please refer to the NCD at <https://www.cms.gov/medicare-coverage-database/details/ncd-details.aspx?NCDId=372&ncdver=1&bc=AAAAQAAAAAAA> for full coverage details.

LungSeq®

Testing for genetic alteration in these genes can determine targeted therapy options that have the potential to decrease tumor burden, decrease symptoms, increase survival, and dramatically improve the quality of life for patients with specific genetic alterations.

Please refer to A52986, Billing and Coding: Biomarkers for Oncology regarding coding and billing information.

3. Melanoma

- BRAF - PRED of response to Vemurafenib
- KIT - PRED of response to Imatinib (TKI)
- NRAS - PROG + PRED for anti-MEK inhibitor

4. Uveal Melanoma

- GNAQ – PROG
- GNA11 - PROG

5. Brain

- BRAF - PRED
- EGFR - PRED
- MGMT - PRED
- IDH1 - DX + PROG

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- IDH2 - DX + PROG
- PIK3CA - PRED
- PTEN - PRED
- CIMP - PRED
- TERT - DX

6. Thyroid

- BRAF - DX + PRED
- KRAS - PRED for Selumetinib
- HRAS - PRED for Selumetinib
- NRAS - PRED for Selumetinib
- PIK3CA - PRED
- RET - DX
- PAX8/PPARG- DX

ThyraMIR Thyroid miRNA classifier (aPCR based microRNA gene expression classifier) (PRED) evaluates the expression levels of 10miRNA genes within an FNA biopsy: miR-29b-1-5p, miR-31-5p, miR-138-1-3p, miR-139-5p, miR-146b-5p, miR-155, miR-204-5p, miR-222-3p, miR-375, and miR-551b-3p.

Oncology Thyroid, provides gene expression analysis of 142 genes utilizing fine needle aspirate, algorithm reported as a categorical result (Afirma - PRED).

ThyraMIR is used as a companion test to ThyGeNEXT when ThyGeNEXT results are inconclusive.

- ThyraMIR, ThyGeNEXT and Afirma services will be considered reasonable and necessary for patients with any of the following conditions:
 - An indeterminate pathology on fine needle aspiration
 - Patients with one or more thyroid nodules with a history or characteristics suggesting malignancy such as:
 - Nodule growth over time
 - Family history of thyroid cancer
 - Hoarseness, difficulty swallowing or breathing
 - History of exposure to ionizing radiation
 - Hard nodule compared with rest of gland consistency
 - Presence of cervical adenopathy
- RosettaGX Reveal thyroid MicroRNA test, an assay used for the classification of indeterminate thyroid nodules, will be considered reasonable and necessary when the conditions outlined above for ThyraMIR, ThyGeNEXT and Afirma are met.
- ThyroSeq is a test utilized to better define the need for thyroid surgery and the type of such surgery. ThyroSeq will be considered reasonable and necessary when the conditions outlined above for ThyraMir, ThyGeNEXT and Afirma are met.

7. Ovary/Fallopian Tube/Peritoneum

- AKT1 - PRED for PI3K/AKT/mTOR inhibitors

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- BRAF - DX + PROG
- KRAS - DX + PROG
- MLH1 promoter hypermethylation - DX
- MSI by PCR - DX
- PIK3CA - PRED for PI3K/AKT/mTOR inhibitors
- PTEN - PRED for PI3K/AKT/mTOR inhibitors
- TP53 - DX + PROG

8. Uterus

- AKT1 - PRED for PI3K/AKT/mTOR inhibitors
- BRAF - PRED
- KRAS - PRED
- MLH1 promoter hypermethylation - DX
- MSI by PCR - DX
- PIK3CA - PRED for PI3K/AKT/mTOR inhibitors
- PTEN - PRED for PI3K/AKT/mTOR inhibitors + DX + PROG
- TP53 - DX + PROG

9. Urinary Tract

- FGFR3 - PROG
- MSI by PCR - DX
- MLH1 promoter hypermethylation - DX

10. Prostate

- The PROGENSA® PCA3 Assay (PRED) is an FDA-approved, automated molecular test (assay) that helps physicians determine the need for repeat prostate biopsies in men who have had a previous negative biopsy.
- PTEN – PROG and THER
- RB1 – DX and PROG
- TP53 - PROG

11. Gastrointestinal Stomal Tumor

- KIT - PRED for Sumatinib + DX
- PDGFRA - PRED for Sumatinib + DX

12. Cancer of Unknown Primary (CUP)

Molecular testing, using the Rosetta Cancer Origin Test™ (PROG), is considered reasonable and necessary in the pathologic diagnoses of CUP when a conventional surgical pathology/imaging work-up is unable to identify a primary neoplastic site. Other applications of this technology are considered not reasonable and necessary and are considered investigational in the use of diagnosis of specific tumor types such as NSCLC and renal cancers.

TUO CTID (Cancer Type ID) (DX) is considered reasonable and necessary in the pathologic diagnoses of CUP when a conventional surgical pathology/imaging work-up is unable to identify a primary neoplastic site. Other applications of this technology are considered not reasonable and

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necessary and are considered investigational in the use of diagnosis of specific tumor types such as NSCLC and renal cancers.

13. Leukemias and Lymphomas

- Acute lymphoid leukemia (ALL)
 - JAK1
 - JAK2
 - BCR/ABL1 - DX
 - ABL1 (kinase domain) - PROG
 - IGH - DX
 - TCRB - DX
 - TCRG - DX
 - TP53 - PROG
 - MLL/AF4 - DX
 - E2A/PBX1 - DX
 - ETV6/RUNX1 - DX
 - KRAS
 - NRAS
 - NOTCH1
 - FBXW7
- Acute myeloid leukemia (AML, and including acute promyelocytic leukemia): All PROG, except where noted below.
 - TP53
 - PML/RARA - DX
 - RUNX1/RUNX1T1 - DX
 - CBFB/MYH11 - DX
 - FLT3 ITD
 - FLT3 D835
 - NPM1
 - KRAS
 - NRAS
 - KIT
 - CEBPA
 - IDH1
 - IDH2
 - DNMT3A
 - JAK2 (p.V617F)
 - JAK2 (exon 12)
 - MPL
 - DEK/CAN - DX

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- ASXL1
 - EZH2
 - TET2
 - PML/RARalpha
 - U2AF1
 - SRSF2
 - ZRSR2
- Hairy cell leukemia
 - BRAF
 - IGH somatic hypermutation - PROG
 - IGH - DX
- Aplastic anemia
 - TCRB - DX
 - TCRG - DX
- Burkitt's lymphoma
 - IGH - DX
 - TP53 - PROG
- Myeloproliferative diseases (MPD - essential thrombocytosis [ET], myelofibrosis & polycythemia vera [PV])
 - KIT
 - TP53
 - BCR/ABL1 - DX
 - JAK2 (p.V617F) - DX
 - JAK2 (exon 12) - DX
 - MPL - DX
 - CALR - DX
 - CSF3R - DX
 - ASXL1 - PROG
 - TET2 - PROG
 - EZH2 - PROG
 - Calr (exon 9)
- Chronic myeloid leukemia (CML) and chronic myelomonocytic leukemia (CMML)
 - ABL1 T3151 – CML only
 - KRAS - PROG
 - NRAS - PROG
 - BCR/ABL1 - DX
 - ABL1 (kinase domain) - PRED for Imatinib
 - FLT3 ITD - PROG
 - FLT3 D835 - PROG
 - KIT - PROG
 - JAK2 (p.V617F) - PROG

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- JAK2 (exon 12) - PROG
- Chronic lymphoid leukemia (CLL)
 - IGH - DX
 - IGH somatic hypermutation - PROG
 - TP53 - PROG
 - IGH direct probe method
 - BTK
 - PLCG2
 - BIRC3
 - BTK
 - NOTCH1
 - SF3B1
- Follicular lymphoma
 - IGH/BCL2 - DX
- Hypereosinophilia Syndrome (HES)
 - KIT (including p.D816V) - PROG + DX
 - FIP1L1/PDGFRA Fusion - DX
- Mantle cell lymphoma
 - CCND1/IGH - DX
- Mastocytosis
 - KIT (including p.D816V) - PROG + DX
 - FIP1L1/PDGFRA Fusion - DX
 - TCRG - DX
- T-cell prolymphocytic leukemia
 - JAK1
 - JAK3
 - TCRB - DX
 - TCRG - DX
- T-cell large granular lymphocytic leukemia(TLGLL)
 - STAT3
 - STAT5B
- Myelodysplastic syndrome (MDS): All below biomarkers are PROG.
 - FLT3 ITD
 - FLT3 D835
 - NPM1
 - KRAS
 - NRAS
 - KIT
 - CEBPA
 - IDH1

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- IDH2
- DNMT3A
- JAK2 (p.V617F)
- JAK2 (exon 12)
- MPL
- ASXL1
- EZH2
- TET2
- Cytogenomic microarray analysis, or alternatively, a single nucleotide polymorphism (SNP) array for the same testing, is covered for the identification of various mutations. These tests are used in the diagnosis/prognosis of various hematological malignancies.
- Waldenstrom's Lymphoplasmacytic Lymphoma
 - MYD88

14. Myeloma Gene Expression Profile (MyPRS) (PROG) isolates plasma cells from myeloma patients, extracts DNA, which is then subjected to MicroArray testing and application of validated software programs to identifying patterns of genetic abnormalities. Seventy highly predictive genes have been identified and correlated to myeloma early relapse. MyPRS gives a predictive risk signature as high-risk or low-risk at this time. A high risk score predicts a less than 20% three-year complete remission whereas a low-risk predicts a five-year complete remission of greater than 60%. The predictive value for the stratification of therapeutic interventions allows these patients to be treated in a more personalized manner based on their own genetic profile.

This test is considered reasonable and necessary only after the initial diagnosis of multiple myeloma has been made and will be available to be used in the stratification of therapeutic interventions. It would be inappropriate to use this test as a diagnostic tool or as a monitoring device of ongoing therapy. Other testing is available for this function.

The coverage is set to include only two clinical settings:

- Once after initial diagnosis is made. In the event MyPRS was not tested at diagnosis of myeloma and there is ongoing initial therapy with persistent disease, MyPRS can be done still as an initial test.

OR

- If relapse has occurred and a change in the therapeutic modalities is contemplated.

Please refer to the limitations section of this policy for frequency limitations.

15. Hereditary neuroendocrine tumor disorders - Must include 6 genes with genomic sequence analysis NEX GEN including:

-
- MAX
- SDHB
- SDHC
- SDHD

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- TMEM127
- VHL

Please refer to the limitations section of this policy for frequency limitations.

16. Hereditary neuroendocrine tumor disorders; duplication/deletion analysis panel - must include analysis for:

-
- SDHB
- SDHC
- SDHD
- VHL

17. Neuroendocrine Tumors

-
- MGMT - PROG
- PTEN – PROG and THER
- RB1 – DX and PROG
- TP53 – DX and PROG
- TSC2 - PROG

18. Prosigna breast cancer gene signature assay (PROG)

Background

Women with early breast cancer and up to 3 locally positive lymph nodes whose tumor is estrogen-receptor positive will usually receive anti-hormonal therapy such as tamoxifen or aromatase inhibitors. U.S. (NCCN) and international (St. Gallen) guidelines predicate the decision for adjuvant chemotherapy on the size and grade of the breast cancer and other factors including genomic assays that provide additional information on risk of recurrence (Hernandez-Ava et al., 2013). According to a 2014 review, "Prognostic factors provide an indication of whether a patient needs subsequent therapy." (Paoletti & Hayes, 2014). Similarly, another 2014 review article states, "Efforts should be focused on reducing chemotherapy in patients unlikely to benefit." (Rampurwala et al., 2014).

The PAM50 breast cancer gene signature test was developed in the late 1990s and initial studies showed a strong correlation with breast cancer recurrence and with complete pathologic response to neoadjuvant chemotherapy (Parker et al., 2009). While test results are reported on a scale of 1-100 as a Risk of Recurrence (ROR) score, the underlying algorithm is also able to classify cases into the luminal A and B, Her2neu, and triple-negative subtype classifications.

The Nanostring nCounter® nucleic acid analysis system replicates the PAM50 algorithm, as an FDA cleared kit, the Prosigna Breast Cancer Gene Signature Assay (FDA, 2013). The Prosigna package insert was most recently updated in January, 2015 (FDA, 2015) reflecting additional studies (Sestak et al., 2014). Notably, the Prosigna platform and the original PAM50 platform have a 0.997 correlation (Dowsett et al., 2013).

For the FDA, the Prosigna test was validated in a large population of post-menopausal, estrogen-receptor positive women based on 1,017 cases of the TransATAC study (Dowsett et al., 2013). The study showed a strong correlation

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with long-term breast cancer recurrence and added substantial additional prognostic information over a clinical treatment score based on standard clinical variables. This study was replicated in an independent population, also on the Prosigna test, using 1,620 samples from the ABCSG8 trial (Gnant, 2014). A separate analysis of these trials validated prediction of distant recurrence in years 5-10 after initial diagnosis (Sestak et al., 2014) and has been incorporated in the FDA labeling (FDA, 2015). The Prosigna test is issued as separate reports, consistent with FDA review and labeling, for node-negative and node-positive (1-3 node) populations. Analytic performance, precision, reproducibility, and analysis of the clinical validations are provided in the FDA labeling (https://www.accessdata.fda.gov/cdrh_docs/reviews/K130010.pdf).

Clinical utility of this breast cancer gene signature has also been assessed. The study of Martin et al. (2015) showed a 20% decision impact on decisions for or against adjuvant chemotherapy in an all-comers population of 200 new cases of incident breast cancer, when Prosigna test information became available after all other clinical information had been considered. The net rates of selecting adjuvant chemotherapy for low, intermediate, and high risk cases was similar to that observed in a meta-analysis of Oncotype DX decision data (Carlson & Roth, 2013). Additional support for the use of these test results in treatment decisions comes from Parker et al. (2009), in which there was a strong association with neoadjuvant chemotherapy response. Low-scoring cases have a very low chance of complete pathological response to neoadjuvant chemotherapy, while high-scoring cases approach a 50% chance of complete pathological response. The same findings have been observed for other breast cancer gene signatures based on prognostic algorithms (Chang et al., 2008).

The Prosigna test is reasonable and necessary when performed according to the FDA label (https://www.accessdata.fda.gov/cdrh_docs/reviews/K130010.pdf).

19. Desmoid Fibromatosis

-
- CTNNB1 – DX and PROG

20. Hepatic Adenoma

-
- CTNNB1 – DX and PROG

21. Bladder

-
- CDKN2A – PROG
- FGFR1 – PROG
- FGFR3 – PROG
- MTOR – PROG
- PIK3CA – DX and PROG
- PTEN – PROG
- RB1 – PROG
- TP53 - PROG

NON-MOLECULAR ASSAYS

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1. The VeriStrat® assay is a mass spectrophotometric, serum-based predictive proteomics assay for NSCLC patients, where “first line” EGFR mutation testing is either wild-type or not able to be tested (e.g., if tissue might not be available). This test is a driver of therapy, most notably EGFR inhibitors such as erlotinib, and it has been validated by randomized controlled studies (Carbone et al. and Stinchcomb et al.) and physician uptake data (Akerley et al.) to support this particular coverage niche.
2. This LCD does not address ALK and ROS1 FISH assays, which are indicated as predictive biomarkers for Crizotinib therapy, since they are currently covered assays. However, it is expected that non-molecular testing for these two biomarkers should provide adequate predictive information.
3. FISH tests for bladder cancer are complex tests based on precision reagents, controls, and mathematical algorithms, all of which must be validated in clinical trials in order to support cutoff points for critical patient care decisions. Therefore, in each local physician’s office or laboratory, this category of testing is not easily replicated by miscellaneous research use or ASR reagents. Novitas will consider the coverage of FISH test kits based on peer-reviewed literature and approved manufacturer claims.
4. Although multiple bladder cancer FISH tests may be covered according to the above general criteria, UroVysion™ Bladder Cancer Kit (UroVysion™ Kit) will be considered medically reasonable and necessary only when performed according to the FDA label (https://www.accessdata.fda.gov/cdrh_docs/pdf3/P030052b.pdf) as follows:

The UroVysion Bladder Cancer Kit (UroVysion™ Kit) is designed to detect aneuploidy for chromosomes 3, 7, 17, and loss of the 9p21 locus via fluorescence in situ hybridization (FISH) in urine specimens from persons with hematuria suspected of having bladder cancer. Results from the UroVysion Kit are intended for use, in conjunction with and not in lieu of current standard diagnostic procedures, as an aid for initial diagnosis of bladder carcinoma in patients with hematuria and subsequent monitoring for tumor recurrence in patients previously diagnosed with bladder cancer.

5. The OVA1™ proteomic assay (PROG) will be considered reasonable and necessary when performed according to the FDA label (https://www.accessdata.fda.gov/cdrh_docs/pdf15/K150588.pdf).
6. The Risk of Ovarian Malignancy Algorithm (ROMA™) is a qualitative serum test (PROG) that combines the results of HE4 EIA, ARCHITECT CA 125 II™ and menopausal status into a numerical score. ROMA™ is intended (per FDA clearance) to aid in assessing whether a premenopausal or postmenopausal woman who presents with an ovarian adnexal mass is at high or low likelihood of finding malignancy at surgery. ROMA™ will be considered reasonable and necessary for women who meet the FDA labeling criteria (https://www.accessdata.fda.gov/cdrh_docs/pdf10/K103358.pdf).

Limitations

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July 2024



**SONIC HEALTHCARE
USA**

Client Communication

Note: Please refer to the indications for any restrictions specific to the various assays. Please see NCD 90.2 for coverage details related to Next Generation Sequencing (NGS) for Patients with Advanced Cancer.

1. Most genomic testing should be a once in a lifetime test. Documentation in the medical record should clearly support the need for repeat testing to include the following: recurrence of disease, change in behavior of disease, etc.
2. The following tests will all be covered once per lifetime per beneficiary:
 - Brain Molecular Biomarkers
 - Hereditary neuroendocrine tumor disorders
 - Hereditary neuroendocrine tumor disorders; duplication/deletion analysis
 - ThyraMIR, Afirma, ThyGeNEXT, RosettaGX Reveal and ThyroSeq tests
 - Should the unlikely situation of a second, unrelated thyroid nodule with indeterminate pathology occur, coverage may be considered upon appeal with supporting documentation
 - TUO CTID (Cancer TYPE ID)
3. While some biomarkers have utility for testing once per lifetime, there are some tumor specific scenarios where repeat testing would be needed for assessment of response to therapy or to identify basis of disease progression. In cases with metastatic or recurrent tumors, repeat testing may be useful in determining further clinical management. Also, biomarkers such as BCR-ABL1 fusion, PML-RARA fusion are useful in monitoring response to therapy and predict a response up to four times per annum.

Notice: Services performed for any given diagnosis must meet all of the indications and limitations stated in this policy, the general requirements for medical necessity as stated in CMS payment policy manuals, any and all existing CMS

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Client Communication

L35396: Biomarkers for Oncology

CPT: 81120, 81121, 81170, 81175, 81176, 81206, 81207, 81208, 81210, 81218, 81219, 81233, 81235, 81236, 81237, 81245, 81246, 81261, 81262, 81263, 81270, 81272, 81273, 81275, 81276, 81287, 81292, 81293, 81294, 81301, 81305, 81310, 81311, 81313, 81314, 81315, 81316, 81320, 81321, 81322, 81323, 81327, 81334, 81340, 81342, 81345, 81347, 81348, 81351, 81352, 81353, 81357, 81360, 81435, 81436, 81437, 81438, 81445, 81450, 81479, 81503, 81520, 81525, 81538, 81540, 81546, 88120, 88121

The ICD10 codes listed below are the top 40 diagnosis codes Sonic most commonly receives from ordering physicians for this limited-coverage test. Medicare supports provider utilization of all the diagnosis codes listed below, except those notated in bold. **If you are providing a diagnosis code that is bolded below, please submit a valid ABN form with the order.** To view the complete policy and the full list of medically supported diagnosis codes, please refer to the CMS website for guidance: <https://www.cms.gov/medicare-coverage-database/search.aspx>

Covered Codes	
C65.2	Malignant neoplasm of left renal pelvis
C66.1	Malignant neoplasm of right ureter
C67.1	Malignant neoplasm of dome of bladder
C67.2	Malignant neoplasm of lateral wall of bladder
C67.8	Malignant neoplasm of overlapping sites of bladder
C67.9	Malignant neoplasm of bladder, unspecified
C68.9	Malignant neoplasm of urinary organ, unspecified
C92.10	Chronic myeloid leukemia, BCR/ABL-positive, not having achieved remission
C92.11	Chronic myeloid leukemia, BCR/ABL-positive, in remission
D45	Polycythemia vera
D47.3	Essential (hemorrhagic) thrombocythemia
D49.4	Neoplasm of unspecified behavior of bladder
R31.29	Other microscopic hematuria
Non-Covered Codes, ABN Required	
D51.9	Vitamin B deficiency anaemia
D53.9	Nutritional anaemia, unspecified
D64.9	Anemia, Unspecified
D72.829	Elevated white blood cell count, unspecified
D75.1	Secondary polycythemia
D75.839	Thrombocytosis, unspecified
D75.9	Disease of blood and blood-forming organs, unspecified

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Non-Covered Codes, ABN Required	
D77	Other disorders of blood and blood-forming organs in diseases classified elsewhere
D89.40	Mast cell activation, unspecified
E11.9	Type 2 diabetes mellitus without complications
E53.8	Deficiency of other specified B group vitamins
E55.9	Vitamin D deficiency, unspecified
E60	Dietary zinc deficiency
I10	Essential (primary) hypertension
I25.10	Atherosclerotic heart disease of native coronary artery without angina pectoris
I48.91	Unspecified atrial fibrillation
I82.512	Chronic embolism and thrombosis of left femoral vein
J45.40	Moderate persistent asthma, uncomplicated
K21.9	Gastro-oesophageal reflux disease without oesophagitis
L20.9	Atopic dermatitis, unspecified
N39.0	Cystitis, unspecified
R31.0	Gross hematuria
R31.21	Asymptomatic microscopic hematuria
R31.9	Hematuria, unspecified
Z08	Encounter for follow-up examination after completed treatment for malignant neoplasm
Z79.899	Other long term (current) drug therapy
Z85.51	Personal history of malignant neoplasm of bladder

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